

AMANA PHARMA SDN BHD

EPHARMA PLATFORM STRATEGY 2026

Strategic Board Brief

Prepared: June 2026

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1. PURPOSE OF THIS BRIEF

This brief outlines the strategic rationale, market opportunity, recommended approach, and 90-day action plan for Amana Pharma Sdn Bhd to enter Malaysia's ePharma (digital pharmacy and telehealth) market. The recommendation is to launch a phased ePharma platform that leverages Amana's existing distributor position to capture segments that pure-play digital health startups cannot serve.

2. ABOUT AMANA PHARMA — CURRENT POSITION

Amana Pharma Sdn Bhd is a pharmaceutical trading company and wholesaler committed to improving access to high-quality, compliant medicines for hospitals, healthcare providers, and institutional buyers. The company specialises in sourcing, supplying, and coordinating access to both registered and special-access medicines, serving clinical needs with efficiency, transparency, and regulatory integrity.

Current Profile

- **Therapeutic scope:** General/multi-category, Oncology/Cancer, Chronic NCDs, Rare/Specialty diseases
- **Customer base:** Hospitals, medical centres, and institutional healthcare buyers

- **Logistics model:** Outsourced delivery partners
- **Core competency:** Regulatory-compliant pharmaceutical sourcing, supply, and special-access medicine pathways

Key Assets Amana Brings to ePharma

- Deep institutional relationships with hospital pharmacists and procurement officers
- Drug pricing and formulary intelligence across all therapeutic categories
- Special-access medicine regulatory expertise
- Established NPRA and MOH compliance culture
- Broad multi-category therapeutic portfolio
- Existing cash flow from distribution business to fund platform investment

3. THE MALAYSIAN EPHARMA OPPORTUNITY

Market Size and Growth

Metric	Figure
Malaysia Digital Health Market (2024)	USD 534M – USD 1.1B
Projected market size (2029)	USD 856 million
Digital health CAGR (2024–2029)	9.89% per year
Telehealth market CAGR (2025–2031)	14.5% per year
Government digital health investment (since 2020)	RM 1.2 billion
National EMR rollout target	2026

Disease Burden Creating Structural Demand

Two-thirds of Malaysians have at least one non-communicable disease (NCD):

- 16% of Malaysian adults have diabetes
- 29% have hypertension
- 33% have high cholesterol
- 2.3 million adults live with three or more concurrent NCDs
- Nearly three-quarters of deaths in Malaysia are attributable to NCDs

This creates enormous, recurring demand for prescription medications. Chronic patients require monthly or quarterly prescription renewals — making them the highest-lifetime-value customers for any ePharma platform.

Regulatory Clarity

The MOH Online Healthcare Services Guidelines 2025 have established a clear legal framework for ePharma operations in Malaysia. Compliance pathways are well-defined, reducing regulatory risk for new entrants who follow the rules from day one.

4. COMPETITIVE LANDSCAPE

The Malaysian ePharma market is active but fragmented. No single player offers an end-to-end integrated solution spanning B2B supply chain, consumer teleconsultation, chronic disease management, and institutional supply.

Player	Strengths	Key Weakness
HeyDoc Health	FutureK capitated model, enSURE drug pricing tool, insurer partnerships	No supply chain ownership; third-party pharmacy dependent
DoctorOnCall	1.9M users, 500+ GP clinics, Samsung Ventures & Khazanah backed	Limited chronic disease management depth
Naluri	USD 19.9M funded, deep chronic disease & mental health coaching	No pharmacy or dispensing capability
Speedoc	USD 28M funded, virtual hospital model, agentic AI	Singapore-centric; premium pricing
BookDoc	MOH-endorsed, 1M+ users, 5 countries	Limited prescription medicine depth
HealthMetrics	1,000+ corporate clients, 4,000+ provider network	No pharmacy component
Alpro ePharmacy	Malaysia's largest prescription chain, 2-hour KL delivery	No B2B institutional supply capability

Critical market gap: No player currently offers an integrated B2B digital supply chain platform for institutional pharmaceutical procurement. This is Amana's natural first move.

5. AMANA'S STRUCTURAL ADVANTAGE

Supply chain ownership — Direct medication fulfillment without third-party pharmacy margins. Lower per-prescription costs and higher stock reliability — critical for chronic patients who cannot afford supply interruptions.

Institutional relationships — Existing hospital pharmacists and procurement officers are the natural first users of a B2B digital ordering platform. Zero new customer acquisition cost for Phase 1.

Drug pricing intelligence — Experience across multiple therapeutic categories provides the data foundation to build an enSURE-equivalent formulary pricing tool, sellable as SaaS to insurers and TPAs.

Special-access medicine expertise — Amana can serve segments including oncology and rare diseases that ePharma startups cannot legally or operationally address. A protected competitive niche.

Regulatory compliance culture — NPRA compliance, MOH relationships, and import/export regulatory experience are already embedded. Extending this to MOH OHS 2025 is an incremental step.

Cash flow advantage — Amana can fund Phase 1 from existing distribution revenues, retaining full equity control without external fundraising.

6. THERAPEUTIC AREA STRATEGY

Chronic NCDs — Consumer ePharma Play (Primary Revenue Opportunity)

Diabetes, hypertension, dyslipidemia, and cardiovascular medicines are the core of every ePharma platform in Malaysia. Standard oral medications, no cold chain requirement, suitable for home delivery, requiring only GP-level teleconsultation — fully permitted under MOH OHS 2025. Chronic patients need monthly repeat prescriptions, creating the most predictable recurring revenue in healthcare. By supplying at distributor cost, Amana eliminates the retail pharmacy margin layer.

Oncology and Rare/Specialty Diseases — B2B Specialty Distribution Platform

These medicines require specialist prescriptions, often involve cold chain storage, and are excluded from online consumer healthcare services under MOH OHS 2025. The right play is a **B2B Specialty Digital Platform** — digital ordering,

special-access medicine tracking, permit management, and formulary tools for existing hospital and oncology centre clients.

General Multi-Category — B2B Digital Supply Chain Foundation

The digital ordering portal covers the entire general product portfolio and is the foundation on which all other platform modules are built.

7. MOH OHS 2025 COMPLIANCE REQUIREMENTS

Requirement	Current Status	Action Required
Malaysian-incorporated company	✓ Met	None
Physical place of business in Malaysia	✓ Met	None
Registered Medical Practitioner in senior management or board	✗ Not yet met	Recruit or appoint licensed doctor to board or senior role
Registered Pharmacist in senior management (for e-pharmacy)	✗ Not yet met	Recruit or appoint licensed pharmacist to management
Secure e-prescription digital flow	To be built	Platform development requirement
No controlled drug delivery by post	To be enforced	System-level restriction in platform
PDPA-compliant data architecture	Partial	Formal compliance audit and documentation needed

Important: Phase 1 (B2B digital supply chain portal) does not trigger MOH OHS 2025 requirements — it is a B2B trading platform, not a healthcare service. The doctor and pharmacist appointments are only required before Phase 2 consumer ePharma launches, giving Amana time to complete these in parallel.

8. RECOMMENDED PLATFORM STRATEGY — THREE PHASES

Phase 1: B2B Digital Supply Chain Platform (Months 0–6)

Objective: Digitise and strengthen the existing institutional distribution business.

What to build:

- Digital ordering portal for existing hospital and institutional clients

- Special-access medicine tracker — permit status, documentation, estimated arrival
- Drug pricing intelligence dashboard and formulary benchmarking tool
- Integration with outsourced logistics for delivery tracking
- Digital invoicing and automated approval workflows

Why start here: No new regulatory licensing required. Existing client base means zero acquisition cost. Generates revenue immediately. Builds the data layer for all future modules.

Investment: RM 500,000 – RM 1,500,000 | **Payback:** 12–18 months

Phase 2: Chronic NCD Consumer ePharma Platform (Months 6–18)

Objective: Build a consumer-facing ePharma platform for repeat prescription management in chronic disease.

What to build or partner for:

- Patient-facing mobile app with registration, health profile, and appointment booking
- **Teleconsultation: Partner, do not build.** API-integrate with DOC2US or Teleme. 3–6 months vs 18+ months to build.
- E-prescription flow routed to Amana-connected pharmacy for dispensing
- Medication fulfillment at distributor cost with home delivery
- Chronic care program: repeat prescription scheduling and medication reminders
- Corporate/SME employer portal for HR benefits administration

Target segment: Corporate and SME employee benefit programs first — predictable B2B revenue before direct-to-consumer.

Investment: RM 1,500,000 – RM 3,000,000 | **Payback:** 18–24 months

Phase 3: Specialty Patient Support and Insurance Integration (Months 18–36)

Objective: Build high-margin SaaS and specialty services on top of Phases 1 and 2.

- Insurance/TPA SaaS tool: Drug pricing benchmark API for claims cost management and formulary optimisation

- Specialty patient support: Hospital-to-home coordination for oncology oral and supportive care medicines
- Rural outreach module via extended logistics and teleconsultation network
- Chronic NCD subscription model: Locked-in recurring revenue from monthly repeat prescription plans

Investment: RM 1,000,000 – RM 2,000,000 | **Payback:** 24–36 months
(high-margin recurring SaaS)

9. BUILD VS. PARTNER VS. ACQUIRE

Build — B2B supply chain portal (Phase 1) and insurance SaaS pricing tool (Phase 3). These are core Amana intellectual property and must be owned.

Partner — Teleconsultation layer (Phase 2). Do not recruit GPs or build a doctor network. API-integrate with an existing licensed platform. Amana's differentiation is supply chain and pricing intelligence, not owning doctors.

Acquire (evaluate at Month 18–24)

- **HeyDoc Health** — Attractive for enSURE pricing tool, FutureK capitated model, insurer pipeline, and AI/blockchain infrastructure. Amana's supply chain fills HeyDoc's most critical gap. A combined entity would be the most complete ePharma platform in Malaysia.
- **Teleme** — Smaller and potentially more acquirable. Full-stack teleconsult, e-prescription, and delivery platform that would accelerate Phase 2 significantly.

10. KEY RISKS AND MITIGATION PLAN

Regulatory compliance — HIGH RISK

MOH OHS 2025 requires a licensed doctor and pharmacist in management before e-pharmacy launch.

Mitigation: Begin recruitment immediately in Month 1. These are prerequisite appointments, not optional.

Outsourced logistics not suited to home delivery — HIGH RISK

Current logistics handles bulk B2B hospital deliveries, not individual patient parcels with same-day delivery requirements.

Mitigation: Audit logistics partner before Phase 2 commitment. Onboard specialist last-mile partner if needed.

Capital intensity of consumer app — MEDIUM RISK

Quality consumer mobile app development is expensive and time-consuming.

Mitigation: Complete Phase 1 first. Use Phase 1 revenue to fund Phase 2. Consider white-label app options.

Doctor supply for teleconsultation — MEDIUM RISK

Building a GP network is costly, regulated, and operationally complex.

Mitigation: Do not build. Partner via API. This is a commodity layer.

MMA e-prescription scrutiny — HIGH RISK

The Malaysian Medical Association is pushing for stricter inspections of all e-prescription platforms.

Mitigation: Build strict consultation-before-dispensing protocols from day one.

Maintain full audit logs. Engage proactively with MMA and MOH.

Competitor response — MEDIUM RISK

HeyDoc and DoctorOnCall could respond with price competition or feature acceleration.

Mitigation: Amana's supply chain advantage is structural and cannot be quickly replicated. The B2B institutional channel is entirely uncontested.

11. FINANCIAL SUMMARY

Phase	Investment (RM)	Payback Period
Phase 1 — B2B Digital Supply Chain	500,000 – 1,500,000	12–18 months
Phase 2 — Consumer ePharma	1,500,000 – 3,000,000	18–24 months
Phase 3 — Specialty & Insurance SaaS	1,000,000 – 2,000,000	24–36 months
Total 3-Year Estimate	RM 3,000,000 – RM 6,500,000	—

All three phases can be funded from existing distribution cash flow without requiring external equity or debt financing, subject to Amana's current working capital position.

12. 90-DAY ACTION PLAN

Month 1 — Foundations

- Engage healthcare regulatory legal counsel for MOH OHS 2025 compliance gap assessment
- Survey 20–30 existing hospital and clinic clients to validate digital ordering portal appetite
- Begin search for Registered Pharmacist candidate to appoint to management
- Begin search for Registered Medical Practitioner for board or senior advisory role
- Issue RFP to 2–3 technology vendors for B2B portal development scoping and cost estimates

Month 2 — Decision and Setup

- Present compliance gap assessment and strategic brief to Board for Phase 1 investment approval

- Select technology partner for B2B portal and begin Sprint 1 of development
- Initiate preliminary commercial discussions with DOC2US or Teleme on teleconsultation partnership terms
- Audit outsourced logistics partner against patient home-delivery requirements; decide if second last-mile partner is needed

Month 3 — First Build Milestone

- Appoint Registered Pharmacist to management — regulatory prerequisite for Phase 2
- Formalise Registered Medical Practitioner board or advisory role
- Complete B2B portal prototype with 5–10 existing clients in pilot testing
- Define go-to-market approach and pricing for Phase 2 corporate health benefit program
- Present 6-month progress review to Board with Phase 2 investment decision

13. STRATEGIC CONCLUSION

The Malaysian ePharma market is growing at 14.5% annually. Regulatory clarity is now in place. The chronic disease burden is creating demand that will only increase. And no existing player has built what Amana already has — a trusted supply chain, institutional relationships, drug pricing intelligence, and special-access medicine capability.

Every ePharma platform currently operating in Malaysia is building from the front end toward the supply chain. **Amana starts from the supply chain and builds toward the patient.** This is structurally faster, cheaper, and more defensible.

The recommendation:

1. Launch a B2B digital supply chain platform within 6 months — low risk, high ROI, no new licensing required
2. Partner, not build, for the teleconsultation layer — fast to market and asset-light
3. Target the chronic NCD corporate benefit channel first — predictable B2B revenue before direct-to-consumer

4. Appoint a pharmacist and doctor to management now — regulatory prerequisite, achievable in 2–3 months
5. Evaluate strategic acquisition of HeyDoc or Teleme at Month 18–24 to accelerate consumer brand

Amana Pharma has the potential to become **Malaysia's first distributor-backed, fully integrated ePharma platform** — the only player in the market that controls supply chain, clinical access, and patient delivery end to end. The window is open and the time to act is now.